

HEALTH SERVICES APPEAL AND REVIEW BOARD

PRESENT:

Christine Moss, Vice-Chair, Presiding
Beth Downing, Vice-Chair
Jay Yedvab, Board Member

The 7th day of October 2009 at London, Ontario

IN THE MATTER OF AN APPEAL UNDER SECTION 20(1) of the *Health Insurance Act*,
Revised Statutes of Ontario, 1990, Chapter H.6, as amended

B E T W E E N:

E.H.
-and-

Appellant

THE GENERAL MANAGER,

THE ONTARIO HEALTH INSURANCE PLAN

Respondent

Appearances:

For the Appellant	Ms. L.H.
For the Respondent,	Dr. Hugh Langley
General Manager, OHIP:	

DECISION AND REASONS

I DECISION

1. This is an appeal from a decision of the General Manager of the Ontario Health Insurance Plan (OHIP) refusing to fund an outpatient multidisciplinary assessment for diagnosis and treatment recommendations for chronic pain at the Mayo Clinic in Rochester, Minnesota in August 2008.

2. For reasons more fully explained below, the Health Services Appeal and Review Board (the “Appeal Board”) finds that the treatment is not an insured service under the *Health Insurance Act*.

3. The appeal is denied.

II. BACKGROUND

4. The Appellant is a young woman who has suffered with chronic pain for many years. This has had a significant impact on her physical well-being and has limited her functioning. She used to play competitive soccer and now has difficulty walking around a mall without stopping to rest. She has had to cut back her course load at university from full time to part time because of pain and related fatigue.

5. The Appellant’s pain began in her calves at about age thirteen. She tried massage, chiropractic, acupuncture and physiotherapy, among other things, but her pain persisted. She also had various tests and assessments. In 2006, she had bilateral fasciotomies on her calves (Dr. Willits, orthopaedic surgeon). This procedure did not alleviate her symptoms.

6. The Appellant’s pain migrated to her arms and hands at about age eighteen and increased in intensity.

7. The Appellant’s mother stated that the Appellant saw at least ten specialists in Ontario over the years but none has been able to determine the cause of her pain. The following is a brief overview of the medical information submitted by the Appellant.

8. The Appellant’s family physician referred her to an internal medicine specialist (Dr. J. Wilson, London) whom she saw from December 2006 through July 31, 2007. After many tests and a referral to a neurologist, Dr. Wilson advised the family physician that she had nothing further to add to the management of this investigation.

9. A neurologist assessed the Appellant (Dr. K. L. Kaye, London) in May and June 2007. After ordering several tests and investigations, Dr. Kaye was unable to determine the cause of the Appellant's pain.
10. The Appellant's family physician referred her to another neurologist (Dr. M. Jog in the Movement Disorder Clinic at London Health Sciences Centre) who assessed her in October 2007. He wondered whether she had restless leg type pain syndrome and suggested that she try the drug, Mirapex, and, if that helped, she might be able to reduce her dependency on analgesics.
11. An anaesthetist saw the Appellant at a pain clinic in February 2008 (Dr. P. Morely-Forster, St. Joseph's Hospital, London). This was described as a follow-up visit and so the Appellant had been to this clinic previously. Dr. Morely-Forster stated that no specific diagnosis had been identified. She suggested continuing with physiotherapy, decreasing Tylenol use and using Zytram, as needed. She offered to see the Appellant again if she had a significant exacerbation of pain and she would consider a lidocaine infusion at that time. The clinic note from this visit states that the Appellant saw Dr. Watts (referred to as a rheumatologist elsewhere in the materials) in Montreal. The Appellant's mother stated that the London Pain Clinic referred the Appellant to the Montreal Pain Clinic.
12. Although no supporting records were provided, the Appellant's mother indicated that the Appellant had also had a urological assessment in Ontario in about 2007.
13. The Appellant's mother, a registered nurse, became especially concerned when her daughter was taken off Tylenol 3's and put on Dilaudid, a narcotic, for her pain in about June 2008. She wanted a physician to determine the cause of her daughter's pain before giving her such strong medication. She stated that her family physician did not know where else to refer her in Ontario for a diagnosis.
14. The Appellant's family physician, Dr. T. Ouellet, submitted on behalf of the Appellant, a Prior Approval Application for Full Payment of Insured Out-of-Country Health Services, dated July 2, 2008. Full funding was requested for "diagnostic testing" relative to the diagnosis of

“undiagnosed chronic pain” to be performed as an outpatient at the Mayo Clinic in Rochester, Minnesota, on July 31, 2008. Under “Comments,” Dr. Ouellet stated, “Patient seeks diagnosis of chronic pain. Diagnosis has been unsuccessful after consulting numerous specialists in Ontario ... Treatment may be possible in Ontario after a diagnosis has been made.”

15. The Respondent denied this application on the basis that the treatment received at the Mayo Clinic is performed in Ontario by an identical or equivalent procedure and because there is no evidence that the Appellant must travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage.

16. The Appellant was assessed at the Mayo Clinic where she was diagnosed with chronic pain syndrome and provided with treatment recommendations, which included stopping the Dilaudid and beginning a pain management program. The Mayo Clinic did not indicate the cause of the Appellant’s pain.

17. Since her assessment at the Mayo Clinic, the Appellant has continued to attend pain clinics in London and Montreal where her chronic pain symptoms continue to be investigated and treated.

III. LAW

18. Health insurance in Ontario is governed by the provisions of the *Health Insurance Act*, Revised Statutes of Ontario, 1990, Chapter H.6, as amended (the Act) and any regulations made pursuant to the Act. Section 12 of the Act provides that every insured person is entitled to payment for insured services. The term “insured services” is defined in sections 1 and 11.2 of the Act to include only those services that are prescribed by regulation.

19. For the most part, insured services under the Act are health services rendered in Ontario. Regulation 552 under the Act also provides a scheme for payment of out-of-country health services in two general circumstances: first, when a subscriber is outside Canada and an emergency arises that requires immediate medical treatment; or second, when the services are

rendered outside Canada with the prior approval of OHIP because they are not performed, or there is a delay, in Ontario. In each of these situations, Regulation 552 establishes separate criteria and conditions for determining if a service is an insured service under the Act. Only the prior approval provisions are relevant to this appeal.

20. The relevant parts of section 28.4 of Regulation 552 for the purposes of this appeal are set out below.

28.4(2) Services that are part of a treatment and that are rendered outside Canada at a hospital or health facility are prescribed as insured services if,

(a) the treatment is generally accepted in Ontario as appropriate for a person in the same medical circumstances as the insured person; and

(b) either,

(i) that kind of treatment that is not performed in Ontario by an identical or equivalent procedure, or

(ii) that kind of treatment is performed in Ontario but it is necessary that the insured person travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage.

IV. ISSUES

21. The issue for the Appeal Board on this appeal is whether the outpatient multidisciplinary assessment at the Mayo is an insured service under the Act. Specifically, this requires consideration of the following issues:

1. Is the treatment generally accepted in Ontario as appropriate for a person in the same medical circumstances as the Appellant?
2. Is the treatment one that is not performed in Ontario by an identical or equivalent procedure?
3. If the treatment is performed in Ontario, is it necessary for the Appellant to travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage?

V. REASONS

Is the treatment generally accepted in Ontario as appropriate for a person in the same medical circumstances as the Appellant?

22. The Appellant's family physician indicated by means of ticking a box on the Prior Approval Application that the treatment, namely "diagnostic testing" for the clinical diagnosis of "undiagnosed chronic pain syndrome," is generally accepted in Ontario as appropriate for someone in the Appellant's medical circumstances.

23. The Respondent did not provide any evidence to dispute this.

24. The Appeal Board is satisfied that further testing and assessment for diagnosis and treatment recommendations for chronic pain is generally accepted in Ontario as appropriate for someone in the Appellant's medical circumstances.

Is the treatment one that is not performed in Ontario by an identical or equivalent procedure?

25. The "treatment" performed over four days at the Mayo Clinic consisted of outpatient assessments by specialists in the areas of neurology, rheumatology, urology and pain clinic. Dr. S.P. Liggett, General Internal Medicine at the Mayo Clinic, coordinated these assessments. After these assessments, Dr. Liggett reported that the Appellant's diagnosis was chronic pain syndrome and made the following treatment recommendations: taper off Dilaudid; initiate Cymbalta; Tylenol/aspirin for breakthrough pain; and enroll in the chronic pain management program at the Mayo.

26. The Respondent submitted that further consultations in the areas of neurology, rheumatology, urology and pain clinics are available in Ontario. While the Appellant's family physician referred the Appellant to London specialists in all of these areas before she travelled to the Mayo, the Respondent argued that there was no evidence that the family physician had attempted to refer the Appellant to specialists at other tertiary care centres in Ontario, such as Toronto, Ottawa and Hamilton.

27. The Respondent also submitted that assessments of patients with complex chronic pain syndrome are widely available in Ontario at a number of major pain centres including Wasser Pain Management Centre at Mount Sinai Hospital; Comprehensive Pain Program at Toronto Western Hospital; Rothbart Centre for Pain Care; as well as pain clinics at Cambridge, Chatham, Hamilton, Mississauga and Ottawa. Dr. Langley, for the Respondent, conceded that pain management clinics focus on managing the pain rather than diagnosing the cause of the pain. He also submitted, however, that these clinics do continue to assess for causes, as appropriate. The Appeal Board notes that a clinic note dated February 24, 2009, from Dr. Banner at the London Pain Clinic, confirms this as he was continuing investigations into the cause of the Appellant's pain at this time.

28. The Appellant's mother submitted that specialists at the Mayo offered a "collaborative approach" to assessing and diagnosing the Appellant that was not offered in Ontario. When questioned by Dr. Langley, however, the Appellant's mother conceded that each of the specialists at the Mayo was seen on a separate day and there was no evidence of a meeting of all the specialists together to discuss the Appellant's case. Dr. Langley submitted that Dr. Liggett, internist at the Mayo, acted as a coordinator, referring the Appellant to different specialists on different days, just as a GP would in Ontario. Dr. Langley conceded that in Ontario, a patient might not get to see all of these specialists so quickly and all in one facility, but he submitted, the treatment offered in Ontario is otherwise equivalent to that offered at the Mayo. Upon review of the notes from the Appellant's Ontario specialists, it appears that the Appellant's family physician coordinated referrals to the specialists and it appears that, in general, each specialist was informed of the other's test results and findings along the way.

29. At the hearing, the Appellant's mother submitted that many of the tests performed at the Mayo had already been performed in Ontario, but that the Mayo ruled out Lyme disease, lupus and fibromyalgia. There is no evidence before us, however, that tests for these conditions are not available in Ontario.

30. The Appellant must establish that the treatment performed at the Mayo is not performed in Ontario by an identical or equivalent procedure. The Appellant had seen specialists in

neurology, rheumatology, urology and pain (the same types of specialists seen at the Mayo) in Ontario before attending the Mayo. Although the Appellant's family physician stated that she did not know where else to send the Appellant for a diagnosis, the Appeal Board notes that the Appellant did continue tests and assessments at the London Pain Clinic and the Montreal Pain Clinic upon her return from the Mayo. The Appeal Board is not persuaded, therefore, that the Appellant was unable to seek and obtain further tests and assessments from other specialists in Ontario.

31. Although a collaborative approach might offer a unique advantage in diagnosis and treatment in some cases, in this case, there was no evidence that the approach taken at the Mayo differed from the approach taken in Ontario. There was no evidence that any of the tests and assessments performed at the Mayo are not performed in Ontario. Finally, although not determinative of the issue, the Appeal Board notes that the specialists at both the Mayo and in Ontario recommended a chronic pain management program, even though they had not been able to determine the root cause of the Appellant's pain.

32. Accordingly, the Appeal Board finds that the Appellant has not established that the treatment performed at the Mayo, namely tests and assessments from various specialists for diagnosis and treatment recommendations for chronic pain, is not performed in Ontario by an identical or equivalent procedure.

Is it necessary to travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage?

33. Having determined that the treatment is performed in Ontario by an identical or equivalent procedure, the Appellant must establish that it was necessary for her to travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage.

34. The Appellant's mother stated that she did not want to wait for further appointments with specialists in Canada. Dr. Langley admitted that there likely would be a delay to obtain further appointments with specialists in Ontario. The Appeal Board must determine whether such a

delay, of unspecified length, would result in death or medically significant irreversible tissue damage.

35. The condition set out in section 28.4(b)(ii) requires a medical judgment about a patient's future, in situations where it is very difficult to predict the future. Against this backdrop, the Appeal Board is of the view that "would" does not mean "inevitable"; however, it does require some degree of certainty of the outcome. The word "would" in this context is synonymous with "probably"; thus, the question in this case is whether, in view of the Appellant's circumstances, death or medically significant irreversible tissue damage would *probably* result while waiting for further tests and assessments in Ontario and Montreal.

36. The Appellant's mother argued that, without a diagnosis, one could not be certain that there would be no tissue damage. Further, she relied on the following statement from the Mayo Clinic's pamphlet on chronic pain: "Pain is defined as 'an unpleasant sensory and emotional experience *associated with actual or potential tissue damage.*'" Further, she argued that her daughter had likely suffered damage to her liver from having to take pain medication for such a long time.

37. The Appeal Board acknowledges the extremely difficult circumstances the Appellant has endured for many years. As set out above, however, the legislation requires evidence of more than the mere possibility of medically significant irreversible tissue damage. In this appeal, there is no evidence in any of the medical documentation from Ontario (including the Prior Approval Application) or from the Mayo Clinic suggesting that delay in further tests or investigations would *probably* result in death or medically significant irreversible tissue damage.

VI. DECISION

38. The Appeal Board finds that the treatment is performed in Ontario by an identical or equivalent procedure and that it is not necessary to travel out of Canada to avoid a delay that would result in death or medically significant irreversible tissue damage. The requirements of section 28.4(2) (b)(ii) of Regulation 552 under the Act have not been satisfied and therefore, the treatment is not an insured service.

39. The appeal is denied.

DATED at Toronto this 6th day of July 2010

Christine Moss, Vice-Chair, Presiding

Beth Downing, Vice-Chair

Jay Yedvab, Board Member